NCCP 00209 · VERSION 10A

Your treatment, explained

Gastrointestinal cancers · NCCP 00209 v10a · Document revised 4 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Modified FOLFOX-6

14-day cycle

Source reconciled; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. New chest pain, breathing difficulty, severe diarrhoea, inability to drink, reduced urine, uncontrolled bleeding or a rapidly worsening symptom needs prompt assessment.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Open your CUH contact / treatment card

Sources: nccp · hse

YOUR COMPLETE REFERENCE

Read what you need, when you need it.

Start with the overview, explore the cycle, or go straight to a question. Urgent advice is always available.

- How each drug works
- Your treatment plan
- Your 14-day timeline
- Baseline and cycle-specific checks
- Whole treatment, scans and checks
- Your port and treatment pump
- Tablets, visits and practical planning
- Your indication and treatment aim
- Living with the 46-hour pump
- What the checks are for
- Recognising treatment effects
- Living with treatment
- Words you may hear
- Supportive medicines
- Understanding the evidence
- Questions to ask

Sources

UNDERSTAND YOUR TREATMENT

Your medicines and what they do

Modified FOLFOX-6 combines oxaliplatin, folinic acid and fluorouracil. It can be used after colon-cancer surgery or to treat advanced colorectal, stomach or gastro-oesophageal-junction cancer. The aim and duration depend on the indication; ask which applies to you.

PLATINUM CHEMOTHERAPY

Oxaliplatin

Eloxatin



Links DNA strands

Oxaliplatin makes links within DNA. These interfere with copying and repair, which can stop a cancer cell dividing. Healthy tissues can also be affected.

Platinum enters cell → DNA links form → Copying is disrupted

What to watch for

Cold can trigger tingling or throat discomfort. Tell the team about persistent numbness, difficulty with buttons or walking, or infusion symptoms. New breathing difficulty always needs assessment.

Sources: nccp · patient

CHEMOTHERAPY MODULATOR

Folinic acid

Calcium leucovorin



Strengthens fluorouracil action

Folinic acid helps fluorouracil remain bound to an enzyme needed to make DNA. In this regimen it increases fluorouracil activity and can also increase its unwanted effects. It is not a vitamin supplement to start yourself.

Folinic acid → Stronger enzyme binding → Fluorouracil effect increases

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What to watch for

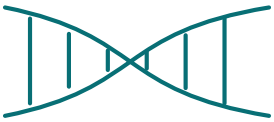
The combined treatment can increase mouth soreness and diarrhoea. Report these promptly. Rare allergic reactions can occur.

Sources: nccp · patient

CHEMOTHERAPY

Fluorouracil

5-FU



Disrupts cell building blocks

Fluorouracil interferes with the production and use of DNA and RNA, the instructions cells need to grow. The infusion extends exposure over approximately two days.

Drug enters cell → DNA / RNA disrupted → Growth slows

What to watch for

Diarrhoea, mouth sores and skin soreness may become serious. Chest pain during the pump infusion needs emergency assessment. Unexpected severe early toxicity can occur even after DPD testing.

Sources: nccp · patient

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Explore your existing anatomy and side-effect map

THE BIGGER TREATMENT PLAN

Your treatment plan

INFUSION + HOME PUMP

14-day treatment cycle

D1 D2 D3 D14

Day 1: oxaliplatin, folinic acid and fluorouracil, followed by a 46-hour fluorouracil pump. A cycle lasts 14 days.

The treating team confirms the indication, schedule and duration. Changes are recorded on your own prescription.

Sources: nccp

BASELINE → TREATMENT → FOLLOW-UP

Checks across the whole treatment course

The columns show the order of care, not equal lengths of time. Confirm who books each check, who reviews it and whom to call if it is missing.

Check	Before starting	Each treatment cycle	Between doses	Decision / response review	Completion / change
Blood count / kidneys / liver	Baseline bloods; DPD before fluoropyrimidine	Repeat before each cycle	Earlier tests if unwell, dehydrated or toxicity develops	Results and recovery guide any delay or dose change	Follow unresolved abnormalities
Symptoms and function	Document existing symptoms and usual activity	Review worst symptoms since the last treatment	Use urgent contacts; do not wait for the next visit	Balance benefit, side effects and priorities	Agree recovery care and ongoing support
Neuropathy / cold sensitivity	Record existing nerve symptoms	Check persistence and effect on daily tasks	Report worsening numbness or cold-triggered throat symptoms	Consultant reviews the oxaliplatin component separately	Nerve symptoms can continue after treatment; arrange review
Heart checks	Review cardiac history; ECG when cardiac function is compromised or clinical findings warrant it. No automatic serial echocardiogram schedule is specified for this regimen.	Ask about new chest pain or breathlessness	Chest pain needs urgent assessment	Repeat heart tests if clinically indicated	Follow any heart problem; no fixed echo series
Response / scans	Confirm cancer setting, disease assessment and intended duration	Review clinical benefit and symptoms	New symptoms may bring review forward	Consultant-directed scans and response assessment	Agree follow-up, completion or another treatment

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00209 v10a · official regimen

THE WHOLE TREATMENT JOURNEY

Modified FOLFOX-6: treatment and checks

Gastrointestinal cancers

Day 1: oxaliplatin, folinic acid and fluorouracil, followed by a 46-hour fluorouracil pump. A cycle lasts 14 days.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

On a small screen, swipe the map sideways. The checks below give the same timing in words.

Treatment, tests and reviews across the course

Care / check	Before starting	Each treatment cycle	Between doses	Decision / response review	Completion / change
Blood count / kidneys / liver NCCP 00209	Baseline bloods; DPD before fluoropyrimidine	Repeat before each cycle	Earlier tests if unwell, dehydrated or toxicity develops	Results and recovery guide any delay or dose change	Follow unresolved abnormalities
Symptoms and function NCCP 00209	Document existing symptoms and usual activity	Review worst symptoms since the last treatment	Use urgent contacts; do not wait for the next visit	Balance benefit, side effects and priorities	Agree recovery care and ongoing support
Neuropathy / cold sensitivity NCCP 00209	Record existing nerve symptoms	Check persistence and effect on daily tasks	Report worsening numbness or cold-triggered throat symptoms	Consultant reviews the oxaliplatin component separately	Nerve symptoms can continue after treatment; arrange review
Heart checks NCCP 00209	Review cardiac history; ECG when cardiac function is compromised or clinical findings warrant it. No automatic serial echocardiogram schedule is specified for this regimen.	Ask about new chest pain or breathlessness	Chest pain needs urgent assessment	Repeat heart tests if clinically indicated	Follow any heart problem; no fixed echo series
Response / scans NCCP 00209	Confirm cancer setting, disease assessment and intended duration	Review clinical benefit and symptoms	New symptoms may bring review forward	Consultant-directed scans and response assessment	Agree follow-up, completion or another treatment

What each check is for

1

Confirm your personal plan

Before starting

Modified FOLFOX-6 combines oxaliplatin, folinic acid and fluorouracil. It can be used after colon-cancer surgery or to treat advanced colorectal, stomach or gastro-oesophageal-junction cancer. The aim and duration depend on the indication; ask which applies to you.

Ask / act: Agree medicines, treatment days, duration and monitoring.

Sources: NCCP 00209

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-04 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.32

Day 1 → Day 14: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Day 1: oxaliplatin, folinic acid and fluorouracil, followed by a 46-hour fluorouracil pump. A cycle lasts 14 days.

Day of cycle	1	2	3	4	5	6	7	8	9	10	11	12	13	14
Day-ward medicines	IV													
Fluorouracil pump	46 hours; disconnect as arranged													
Cold sensitivity	Often early; may last longer													
Blood counts / infection	Timing varies; no guaranteed safe days													

Day of cycle	1	2	3	4	5	6	7	8	9	10	11	12	13	14
Mouth / bowel / energy	Record symptoms throughout													

Treatment and pump: Day 1 treatment is followed by a 46-hour fluorouracil infusion. The team confirms the exact pump disconnection time.

Blood counts: Counts can fall between visits. The day of the cycle and how well you feel cannot rule out infection.

Cold sensitivity: Tingling or throat discomfort can occur early and may persist; follow the precautions your team gives you.

Symptoms over time: Mouth and bowel symptoms or fatigue can overlap. Numbness may accumulate across cycles; describe changes before the next dose.

> **Why these bands stay broad:** Bars show planned treatment and possible symptom patterns, not measured risk. Side effects can overlap and persist. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00209 v10a · official regimen ↗ · eviQ: patient information (supportive explanation; NCCP schedule takes priority) ↗

> **There are no guaranteed "safe days".** Act on fever, sudden illness or a new serious symptom whenever it occurs. Do not wait for the next appointment.

Read the whole cycle as text

Day 1: Infusion and pump connection

Symptoms and blood tests are reviewed. The day-ward medicines are followed by connection of a fluorouracil pump. Tell the nurse immediately about a reaction or pain at the infusion site. Before leaving, confirm the pump-care instructions, disconnection appointment and contact number.

Day 2–3: The pump is delivering treatment

The infusion runs for about 46 hours from connection; the calendar day does not determine the exact finish time. Follow the instructions for keeping the line dry and secure. Avoid cold exposure if it triggers symptoms. Call for a leaking, disconnected or unexpectedly stopped pump. Do not reconnect, alter settings or disconnect it yourself unless specifically trained and instructed.

Day 4–7: After disconnection

Nausea, diarrhoea, mouth symptoms, fatigue or cold sensitivity may continue. Use prescribed support. New symptoms deserve advice even if the pump has finished. Report poor intake, persistent diarrhoea or mouth pain early. Chest pain or major breathing difficulty requires emergency help.

Day 8–14: Between visits and the next review

Blood counts can fall between visits. Numbness may accumulate over cycles. Describe any difficulty fastening buttons, walking or handling objects before more oxaliplatin. Bring the symptom passport and check the next appointment. Day 1 usually repeats 14 days after this cycle started, only if confirmed.

YOUR TREATMENT, STEP BY STEP

Your port and treatment pump

From preparing the line to coming home after disconnection. Your team arranges the appointments and shows you what to do.



1 · Before treatment

Choose and prepare the line

The team explains whether a port or PICC suits your treatment. You receive instructions for the insertion appointment.



2 · Insertion and recovery

A separate appointment may be needed

For a port, numbing medicine is used and a small device is placed under the skin. Ask about sedation, transport and wound care.



3 · Day 1

The nurse connects your pump

After the day-ward medicines, the nurse checks and connects the fluorouracil pump. Ask for a demonstration before leaving.

4 · At home

Treatment continues for about 46 hours

Carry the pump as shown. Keep the dressing dry and protect the line from pulling, heat and damage.

5 · As arranged

Return for disconnection

A trained professional checks the infusion, disconnects the pump and cares for the line. The appointment is based on connection time.

6 · Between cycles

The access device usually stays

A port needle can be removed while the port stays under the skin. A PICC remains in the arm with ongoing dressing and line care.

Port or PICC: what is the difference?

A port is a small chamber beneath the skin, usually on the chest, attached to a tube ending in a large vein. A special needle goes through the skin into the port when it is used. A PICC enters a vein in the upper arm; a short section remains outside under a dressing. Both can deliver a home infusion. Your team helps choose the device and teaches its care.

Before insertion and during recovery

Tell the team about allergies, blood thinners, infections and any other cancer medicines. Follow the instructions about eating, drinking and transport; do not stop medicines yourself. Port placement uses local anaesthetic, sometimes with sedation or general anaesthetic. The team explains the procedure and risks, including infection, bleeding, clots and insertion complications. Ask how to manage discomfort, protect the wound and resume activity. Report increasing pain, redness, swelling, discharge or fever.

Connection: what the nurse does

The nurse checks the line and treatment, secures the connection and dressing, and starts the pump. Before leaving, you should have the disconnection location and time, contact numbers including out of hours, and the written spill instructions. Ask to be shown what normal pump progress looks like. You do not need to calculate a rate.

Carrying it, sleeping and washing

Use the supplied carrier and the position recommended for your pump. Keep tubing free from pulling and kinks. Ask how to place the pump safely for sleep and how to protect the line and dressing when washing. Do not immerse it or swim. Avoid direct heat and cold. An elastomeric pump has a balloon that shrinks slowly and may have no alarm; an electronic pump has different instructions. Follow the instructions for the device you actually receive.

Disconnection is not port removal

Keep the appointment even if the pump looks empty, unless the team changes the plan. Staff check the pump and line and arrange safe disposal. After flushing and other line care as required, the port needle may be removed and a small dressing applied. The implanted port stays in place until a separate removal procedure is arranged. A PICC remains in place and needs scheduled dressing and flushing care.

Leakage, disconnection or unexpected pump progress

Call the team immediately if the pump leaks, is damaged, finishes unexpectedly early, appears not to run, or the line disconnects. Follow the emergency or spill steps you have been specifically taught, including use of a clamp if instructed. Do not reconnect, change settings or disconnect the pump yourself. Keep other people away from leaked medicine. If medicine touches skin, rinse it off and wash with soap and water, then seek advice.

Pain, swelling or feeling unwell

Contact oncology immediately for pain or swelling around the line, a wet/loose dressing, redness, fever, shivering or suddenly feeling unwell. A painful swollen arm or neck needs urgent advice. Severe chest pain, major breathing difficulty or collapse: call 112 / 999. Do not wait for pump disconnection or the next clinic visit.

The drawings show general steps, not instructions for inserting or accessing a line. Your hospital confirms its own arrangements and the care instructions for your device.

Open the complete port and pump guide · **Printable port and pump guide**

Sources: eviQ: port · eviQ: PICC · eviQ: pump. Reviewed against these sources 16 October 2026; local nursing and patient review pending.

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.



STEP 1

Before the first treatment

Confirm the aim, likely course length, blood-test arrangements and a working urgent contact number. DPD testing is required before starting the fluoropyrimidine unless already completed.



STEP 2

Line and pump arrangements

Confirm venous access, the 46-hour fluorouracil pump and disconnection arrangements before leaving. Know whom to contact for leakage, an alarm, line pain or a pump that finishes unexpectedly.



STEP 3

If treatment is delayed

Ask the unit to confirm the new treatment and blood-test dates. Re-anchor supportive medicines to the treatment actually given; do not take an extra or catch-up anticancer dose yourself.



STEP 4

Getting help between visits

Follow your oncology unit’s urgent advice for fever, infection, severe diarrhoea, chest pain or difficulty breathing. Bring your treatment card and current medicines list.

Open practical treatment guide · Printable practical guide

Sources: NCCP 00209 v10a · official regimen. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

Support around the medicines

1 Nausea prevention and rescue

Take the prescribed prevention medicines at the planned times and keep rescue treatment available. Tell the team if nausea or constipation is not controlled; do not duplicate steroid or anti-sickness medicines.

2 Bowel symptoms

Report diarrhoea promptly and follow the written rescue plan. Severe symptoms, fever, dehydration or inability to drink need urgent advice.

3 Counts and recovery

White-cell growth-factor injections are prescribed according to individual risk and previous complications. They are not automatically required for every patient or every cycle.

4 Cold and nerve symptoms

Avoid cold exposure as advised after oxaliplatin. Report persistent numbness or difficulty with buttons, walking or everyday tasks before the next dose.

5 No ice chips with oxaliplatin

Do not suck ice chips during bolus fluorouracil in this regimen: oxaliplatin can cause cold-triggered nerve symptoms. New breathing difficulty still needs clinical assessment.

Sources: nccp

TREATMENT REFERENCE · 01

Your indication and treatment aim

The same medicines can have different aims.

After colon-cancer surgery

Adjuvant treatment aims to reduce recurrence risk after complete resection. The likely absolute benefit depends on stage, pathology, fitness and other features. Ask how recurrence risk and neuropathy risk influence the proposed duration.

Sources: nccp

Advanced colorectal or upper-GI cancer

Treatment may aim to control growth, relieve symptoms and prolong life. The plan is reviewed against response, toxicity and your priorities. A treatment break or a change of components requires an individual discussion.

Sources: nccp

How long will treatment last?

Do not infer duration from the number of passport pages. Ask for the planned number of cycles and review points. The NCCP schedule distinguishes adjuvant colon treatment, metastatic colorectal treatment and upper-GI treatment.

Sources: nccp

TREATMENT REFERENCE · 02

Living with the 46-hour pump

Ask for a demonstration and written contact instructions.

Before leaving

Know what normal pump progress looks like, how to carry it and how to protect the line while washing or sleeping. Check who will disconnect it and when. Have the team check anything you do not understand.

Sources: patient

Leakage or a delivery problem

Use the hospital spill instructions, avoid direct contact with leaked medicine and call the team. Do not repair a connection or adjust the pump. A pump finishing much sooner or later than expected also needs advice.

Sources: patient

Chest pain while on fluorouracil

Fluorouracil can affect the heart. New chest pain during treatment needs urgent assessment; severe pain, collapse or major breathing difficulty means 112/999. Tell emergency staff that fluorouracil is infusing. Follow the team's instructions about the pump.

Sources: patient

TREATMENT REFERENCE · 03

What the checks are for

Symptoms, blood results and the treatment aim answer different questions.

Blood counts

Neutrophils protect against infection, haemoglobin carries oxygen and platelets help clotting. A normal previous result or feeling well does not prove the current counts are safe. Fever or sudden illness needs immediate advice.

Sources: hse

Kidney and liver function

Organ function can change drug exposure. Vomiting or diarrhoea can cause dehydration and worsen kidney function. Report reduced urine, poor intake, jaundice and new medicines. The clinical assessment combines results with symptoms.

Sources: nccp

Describe symptoms in practical terms

Tell the team about the worst symptoms since the last visit, even if improved today. Examples include extra stools above your usual pattern, whether mouth pain affects drinking, and whether numbness affects buttons or balance.

Sources: patient

How benefit is assessed

After surgery, treatment aims to reduce the chance of recurrence; there may be no visible cancer to measure. In advanced disease, scans, symptoms and function help assess control. Ask when review is planned and what would lead to a change.

Sources: nccp

Recognising treatment effects

Early reporting helps the team manage symptoms and protect function.

Acute cold sensitivity and persistent neuropathy

Cold-related tingling can occur soon after oxaliplatin. Persistent numbness between cycles is different and may worsen with repeated treatment. Describe duration and effect on hand use or walking. Some neuropathy can remain after treatment ends.

Sources: patient

Bowel and mouth symptoms

Fluorouracil can affect rapidly renewing tissues. Keep track of stools compared with your normal pattern, including stoma output. Report diarrhoea, painful ulcers or reduced drinking; do not wait for the next cycle if you are dehydrated or unwell.

Sources: patient

Hand-foot soreness and skin changes

Tell the team about red, tender or peeling palms or soles. Reduce friction and use the skin care recommended locally. Pain limiting normal activity needs review. Do not assume it must be tolerated to complete treatment.

Sources: patient

Fatigue and nausea

Use manageable activity, rest and the prescribed anti-sickness plan. New profound weakness, faintness or breathlessness warrants assessment because anaemia, infection, dehydration and other problems can look like ordinary fatigue.

Sources: hse

TREATMENT REFERENCE · 05

Living with treatment

Plan around the activities and people that matter to you.

Food, drink and practical help

Use foods and drinks you can manage. Tell the team about weight loss, poor intake or difficulty shopping and cooking; ask for dietitian or practical support. A fixed daily fluid target is not appropriate for everyone, particularly with heart or kidney problems.

Sources: patient

Activity, rest and driving

Balance rest with manageable activity. Record what fatigue stops you doing. Do not drive when dizzy, unwell or affected by sedating medicines. Ask about work adjustments and transport for treatment days.

Sources: patient

Medicines, vaccines and supplements

Bring a complete medicine list, including over-the-counter products and supplements. Ask the oncology pharmacist before changes. Discuss vaccination timing. Do not stop prescribed long-term medicines on your own.

Sources: hse

Fertility, contraception and breastfeeding

Discuss fertility preservation before treatment when relevant. Ask for a written contraception and breastfeeding plan covering each medicine and the period after treatment. Tell the team promptly about a possible pregnancy.

Sources: hse

Support for you and your family

Ask about psycho-oncology, social work, counselling and symptom support. Take information in manageable amounts. A family member or trusted person can help record questions if you wish. Discuss how treatment fits your priorities.

Sources: patient

TREATMENT REFERENCE · 06

Words you may hear

A short reference for clinic discussions.

Adjuvant, metastatic and cycle

Adjuvant means treatment after the main treatment, usually surgery, to reduce recurrence risk. Metastatic means spread to a distant site. A cycle includes treatment and the interval before the next planned start.

Sources: nccp

DPD, neuropathy and dose modification

DPD is an enzyme involved in fluorouracil breakdown. Neuropathy means nerve damage or dysfunction. Dose modification means changing the amount, timing or components to balance benefit and harm.

Sources: nccp

Understanding the evidence

Ask which evidence matches your cancer, treatment aim and medicines.

Adding oxaliplatin to fluorouracil-based treatment improved long-term outcomes in a study of people with stage III colon cancer after surgery. That study used FOLFOX4, a related but different schedule. It does not predict benefit for every indication of modified FOLFOX6 or for an individual person.

Sources: mosaic

PREPARE FOR THE CONVERSATION

Questions worth asking

Does having more side effects mean treatment is working?

No. Side effects do not reliably show whether the cancer is responding. Report them so they can be treated. Your team assesses benefit using the tests and review plan appropriate to your cancer.

Why might a dose be delayed or reduced?

Your team may allow recovery, investigate a symptom or reduce the chance of lasting harm. Ask which drug changes, why, and when review is planned. A delay does not itself mean treatment has failed.

What should I say when calling?

Give the regimen name, last treatment date, symptoms and when they started. Explain whether you can eat, drink, pass urine and manage usual activities. Give your temperature if available, but do not delay calling to collect details.

What belongs in the consent conversation?

The individual aim and likely benefit, uncertainty, reasonable alternatives including not proceeding, common and serious risks, lasting effects, fertility and practical concerns. You can ask for an interpreter, more time or another explanation and can reconsider your decision.

How many cycles should I use this passport for?

The passport provides reusable recording pages, not a prescribed course length. Your oncology team sets the duration and can provide extra pages.

Can I use this page to decide if blood results are safe?

No. Treatment decisions require the clinical protocol, symptoms, previous toxicity, organ function and the prescriber's review. The patient page explains the checks and does not authorise treatment.

> **Try explaining the plan back:** Can you explain which medicines you are receiving, the schedule, the contact plan and the symptoms that need prompt help?

SOURCE TRANSPARENCY

Where this information comes from

Regimen schedule: NCCP 00209 v10a. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.

- NCCP 00209 v10a · official regimen
- eviQ: patient information (supportive explanation; NCCP schedule takes priority)
- HSE: chemotherapy side effects and urgent advice

- NCI: nausea and vomiting
- MOSAIC: ten-year follow-up (2015)
- eviQ: treatment through an implanted port
- eviQ: treatment through a PICC
- eviQ: elastomeric infusion pump care

SACTCheck · Developed by Dr Paul O'Brien · Sources checked 2026-11-04.
This information supports your treatment discussion. Your oncology team confirms your personal treatment and urgent-care plan.

Written source addresses

https://healthservice.hse.ie/documents/6549/209_v10a_FOLFOX_6_Modified.pdf

https://www.eviq.org.au/medical-oncology/colorectal/metastatic/114-colorectal-metastatic-folfox6-modified-fluo/patient-information

https://www.eviq.org.au/p/507

https://www.eviq.org.au/p/543

https://www.eviq.org.au/p/502

https://www2.hse.ie/conditions/chemotherapy/side-effects/

https://www.cancer.gov/about-cancer/treatment/side-effects/nausea-vomiting

https://pubmed.ncbi.nlm.nih.gov/26527776/